PRODUCT REQUIREMENTS DOCUMENT

Tata 1mg Family Care

Manage your parents' medicines from anywhere, with no app needed on their side.

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Summary

Millions of working adults manage their parents' medicines from another city. They reorder over the phone, upload prescriptions on their parents' behalf, and find out a strip has run out only when a parent mentions it. **Family Care** lets one Tata 1mg account manage medicines for linked family members: see what each person takes, get a warning before supplies run out, and approve refills that are delivered to the parent's address. The parent doesn't need to use the app at all.

12.9%
of Indians are 60 or older (NFHS-6)

~2 in 3
people over 60 live with a chronic condition (LASI)

0 taps
needed from the parent to get a refill delivered

Problem

Chronic conditions such as hypertension and diabetes mean the same medicines every month, often for years. The person who pays and arranges those refills is often an adult child, not the patient. Medicine apps are built for one person ordering for themselves, so caregivers work around them:

1. **Shared logins.** Families share one account, so medicines, records and order history for different people get mixed together.
2. **No early warning.** Nobody knows a supply is about to run out until a dose is missed.
3. **Prescription chasing.** The parent's prescription is on paper, so the caregiver asks for photos over WhatsApp every time.
4. **Delivery confusion.** Orders go to the wrong address, or the parent refuses a delivery they weren't expecting.

Insight: In a chronic-care household the buyer and the user of the medicine are different people. The product should serve the buyer without making the user change anything.

What we know vs. what we're assuming

Statement	Status	How we'd check
India's 60+ population share is rising: 12.9% in NFHS-6, up from 11.8% in NFHS-5.	Known	NFHS-6

About two-thirds of people over 60 have at least one chronic condition; about 23% have more than one.	Known	LASI Wave 1
Many repeat orders on 1mg are placed by someone other than the patient.	Assumption	Compare the account's age with the patient name on the prescription; check orders where the delivery city differs from the account's usual city
Running out of medicine causes missed doses and urgent orders.	Assumption	Count reorders placed 0–2 days after an estimated run-out date; interview 10 caregivers
Parents will accept a caregiver managing their orders.	Assumption	Interviews with 5–6 parent–child pairs

Who it's for

Persona	Situation	What they need
The remote caregiver Karthik, 32, Bengaluru; parents in Coimbatore	Manages his father's BP and diabetes medicines from 360 km away	Never have his father run out, without daily calls about it
The parent Lakshmi, 64, Coimbatore	Takes 4 medicines a day, uses WhatsApp but doesn't shop online	Medicines arrive on time, with no app to learn
The sibling Divya, 29, Pune	Shares the cost and wants to help	See what was ordered and split the bill

Goals and non-goals

Goals

- Prevent run-outs for family members' chronic medicines
- Let a caregiver order for a parent without sharing a login
- Raise repeat-order retention on chronic medicines

Non-goals (v1)

- Dose reminders or adherence tracking for the parent
- Teleconsultations booked by caregivers
- Insurance claims

Success metrics

Type	Metric	Target (hypothesis)
North star	Family members with no run-out in the last 90 days	Set after the pilot baseline
Primary	90-day repeat rate for chronic medicines in Family Care accounts vs. control	+15 percentage points
Primary	Refills approved before the estimated run-out date	≥ 70%
Input	Caregivers who link a family member and add one or more medicines	≥ 40% of those who start setup

Guardrail	Refused or returned deliveries	No rise vs. baseline
Guardrail	Consent complaints or privacy tickets	Zero tolerance; reviewed weekly

Solution

How it works

1. **Link.** The caregiver adds a family member by phone number. The parent confirms by replying YES on WhatsApp or SMS. This consent step is required, can be revoked at any time, and needs no app.
2. **Build the medicine list.** The caregiver uploads the prescription once. A 1mg pharmacist checks it and sets up the list with doses and quantity per pack.
3. **Track supply.** 1mg estimates each medicine's run-out date from the dose and the last delivery. The parent can correct it with a one-tap "I still have some" reply.
4. **Approve refills.** Seven days before a run-out, the caregiver gets one combined refill to approve. Optional auto-approve applies to unchanged prescriptions.
5. **Deliver.** The order goes to the parent's address. The parent gets a WhatsApp message with the delivery time and what's inside.
6. **Renew the prescription.** When a prescription is close to expiry, 1mg prompts the caregiver to renew it, with a teleconsult option where allowed.

Requirements

Priority	Requirement
P0	Family profiles: up to 4 linked members, each with their own address, prescriptions and order history
P0	Consent: the parent opts in by WhatsApp or SMS, can revoke any time, and every caregiver action is logged
P0	Medicine list with pharmacist check and run-out estimate per medicine
P0	Refill approval 7 days before run-out, combining items that run out around the same time
P0	Delivery notice to the parent in their chosen language
P1	Auto-approve for unchanged prescriptions, with a cap on spending
P1	Co-caregiver: a sibling can view the account and split payment
P1	Prescription expiry reminders, with a teleconsult link
P2	Price alerts when a cheaper substitute with the same salt is available, on pharmacist advice

Edge cases and safety

- **Dose change.** If a new prescription changes a dose, auto-approve pauses until a pharmacist confirms the new schedule.
- **Scheduled or controlled drugs.** These are excluded from auto-approve and follow existing prescription rules.
- **Consent withdrawn.** The caregiver loses access straight away. Pending orders are cancelled, and the caregiver is told the link has ended, not given a reason.
- **Hospital stay or travel.** Refills for a family member can be paused with one tap.

Rollout and experiment

1. **Concierge test (4 weeks).** Pharmacists manage refills by hand for 200 caregiver households using WhatsApp. This tests demand, consent and how accurate the run-out estimates are before anything is built.
2. **Pilot (8 weeks).** Build P0 for users whose orders are delivered to an address in a different city from where they place them. Compare 90-day repeat rate against a holdout group.
3. **Scale.** Launch to everyone with chronic-medicine orders. Then add P1 features and co-caregiver payments.

Risks and open questions

Risk	Mitigation
Privacy and consent for health data	Explicit opt-in from the parent, revocable at any time; caregiver actions logged; follow India's data-protection rules for sensitive data
Run-out estimates are wrong (missed doses, extra stock)	The parent can correct them in one tap; estimates learn from actual reorder timing
Auto-approve orders medicine the patient no longer takes	Spending cap, pause on dose changes, and a check with the parent every quarter

Open questions: What share of 1mg's chronic-medicine orders already go to an address in a different city from the account? Will parents reply on WhatsApp reliably enough for consent and corrections? Should co-caregiver payment launch in v1?

Sources

- India's ageing population exposes a health data gap, Policy Circle (NFHS-6 share of people 60+)
- Almost 70% senior citizens in India have a chronic illness, Down To Earth (LASI Wave 1)
- Carers' family accounts, Pharmacy2U (UK example of the model)